

PARTICIPATING PROVIDER AGREEMENT

Commercial and Government Products

by and between

HIGHMARK HEALTH PLAN, INC.

(the "Plan" / "Payer")

and

KEYSTONE HEALTH SYSTEM, INC.

(the "Health System" / "Provider"), contracting on behalf of
its affiliated hospitals, physician organizations, and ancillary facilities

Agreement No. HM-KHS-2025-0417 • Effective January 1, 2025

SPECIMEN / TRAINING DOCUMENT — NOT A REAL CONTRACT. Fictional parties, rates, and terms created to illustrate
managed-care contract structure for the Matrix pricing-engine project.

ARTICLE I — PARTIES, RECITALS & TERM

This Participating Provider Agreement (the “Agreement”) is entered into as of January 1, 2025 (the “Effective Date”) by and between Highmark Health Plan, Inc., a Pennsylvania health insurance corporation (“Plan”), and Keystone Health System, Inc., an integrated delivery network (“Health System”), on behalf of the Covered Entities listed in Exhibit A.

RECITALS.

WHEREAS, Plan arranges for the delivery of health care services to Members enrolled in the benefit plans and products described in Exhibit B; and

WHEREAS, Health System owns and operates, or is affiliated with, the hospitals, employed and affiliated physician organizations, and ancillary service facilities identified in Exhibit A; and

WHEREAS, the parties desire that the Covered Entities furnish Covered Services to Members on the rates and terms set forth herein;

NOW, THEREFORE, in consideration of the mutual covenants below, the parties agree as follows:

1.1 Term and Renewal.

The initial term is three (3) years commencing on the Effective Date. Thereafter the Agreement renews automatically for successive one (1) year terms (“evergreen”) unless either party delivers written notice of non-renewal at least one hundred eighty (180) days before the end of the then-current term. Rate Exhibits (Exhibit C) are amended annually pursuant to Section 4.4.

1.2 Termination.

Either party may terminate without cause on one hundred eighty (180) days’ written notice, and for cause (uncured material breach) on ninety (90) days’ written notice. Members in an active course of treatment are entitled to continuity of care at contract rates for up to ninety (90) days following termination.

ARTICLE II — DEFINITIONS

Allowed Amount. The lower of (i) the rate determined under the applicable Rate Exhibit and (ii) the Provider’s Billed Charges (the “lesser-of” rule), before application of Member cost-sharing.

Billed Charges. The Provider’s undiscounted charges as reflected on a clean claim.

Clean Claim. A claim submitted with all data elements required for adjudication, without defect or need for additional information.

Covered Entity. Any hospital, physician organization, practitioner, or ancillary facility listed in Exhibit A and identified by its Tax Identification Number (TIN) and National Provider Identifier (NPI).

Covered Services. Medically necessary services that are a benefit of the Member’s Product and rendered by a Covered Entity.

DRG. Diagnosis-Related Group, using the CMS Medicare Severity grouper (MS-DRG) version in effect on the date of admission.

Medicare Rate. The amount that would be paid under the applicable CMS fee schedule (MPFS, OPFS, or IPPS) for the same service, locality, and date of service.

Product. A benefit plan or line of business offered by Plan and listed in Exhibit B (e.g., Commercial PPO, Medicare Advantage).

RVU / Conversion Factor. The Relative Value Units and dollar Conversion Factor published in the CMS Medicare Physician Fee Schedule for the applicable year.

ARTICLE III — PROVIDER OBLIGATIONS

3.1 Covered Services. Health System shall cause each Covered Entity to furnish Covered Services to Members without discrimination and in accordance with applicable law and Plan's Provider Manual.

3.2 Roster Maintenance. Health System shall notify Plan in writing within thirty (30) days of any addition, removal, or change to a Covered Entity (Exhibit A), including acquisitions, divestitures, and changes in TIN or NPI. Exhibit A is deemed amended upon Plan's acceptance of such notice.

3.3 Credentialing. Each practitioner must be credentialed under Plan's policies. Rates in Exhibit C apply only to properly credentialed and rostered Covered Entities.

3.4 Claims Submission. Clean Claims must be submitted within one hundred eighty (180) days of the date of service ("timely filing") in the HIPAA-standard 837 format.

ARTICLE IV — COMPENSATION

4.1 Payment in Full. Payment of the Allowed Amount, less applicable Member cost-sharing, constitutes payment in full for Covered Services. Provider shall not balance-bill Members for covered amounts.

4.2 Rate Determination. The Allowed Amount is determined by (a) the Covered Entity and its position in the roster hierarchy (Exhibit A), (b) the Member's Product (Exhibit B), and (c) the care setting and the corresponding methodology in the Rate Exhibit (Exhibit C), subject to the lesser-of rule and the payment policies of Exhibit E.

4.3 Rate Basis. Except where a fixed dollar rate is stated, rates are expressed as a percentage of the applicable Medicare Rate for the stated base year, and are materialized to a concrete fee schedule at adjudication.

4.4 Annual Escalator. Effective each January 1 following the base year, the percentage-of-Medicare rates in Exhibit C increase by three percent (3.0%) compounded annually, unless the parties agree otherwise in a written amendment. A worked example appears in Exhibit C, Section C.6.

4.5 Lesser-of Rule. In no event shall the Allowed Amount for a line exceed the Provider's Billed Charges for that line.

EXHIBIT A — COVERED ENTITIES (ROSTER)

The following entities are Covered Entities under this Agreement. The Allowed Amount is determined by the most specific matching entity, applying the resolution hierarchy below.

A.1 Resolution Hierarchy (most specific wins).

Practitioner-at-Facility → Physician Organization → Facility → Hospital → Health System (IDN parent). Where two entries tie at the same level for the same Product and setting, the claim is escalated for manual review (no rate is guessed).

A.2 Roster.

Entity (Covered Entity)	Type	TIN	NPI	Setting(s)
Keystone Health System, Inc.	IDN Parent	23-1000001	KEYSTONE-NPI01	All (fallback)
Keystone General Hospital	Acute Hospital	23-1000002	KEYSTONE-NPI03	Inpatient / Outpatient
Keystone Children's Hospital	Acute Hospital	23-1000003	KEYSTONE-NPI04	Inpatient / Outpatient
Keystone Cardiology Associates	Physician Org	23-1000004	KEYSTONE-NPI02	Professional
Robert Chen, MD (Cardiology)	Practitioner	—	KEYSTONE-NPI05	Professional @ Keystone Cardiology
Keystone Imaging Center	Ancillary (Imaging)	23-1000006	KEYSTONE-NPI06	Outpatient / Ancillary
Keystone Behavioral Health	Ancillary (BH)	23-1000007	KEYSTONE-NPI07	Per-diem / Professional

A.3 Illustration. A claim from Dr. Chen (NPI05) rendering at Keystone Cardiology resolves at the **Practitioner-at-Facility** level and prices under his specific line in Exhibit C.3. The same CPT billed by an unspecified Keystone Cardiology provider resolves one level up, at the **Physician Organization** line.

EXHIBIT B — PRODUCTS & NETWORK SCOPE

This Agreement applies to the Products checked below. The same roster prices differently by Product; rate anchors are stated as a percentage of the applicable Medicare Rate.

Product (Line of Business)	Network	In Scope	Rate Anchor (prof.)	Facility Anchor
Commercial PPO	Broad	Yes	130% of MPFS	DRG base \$12,000 / APC 250% OPPS
Commercial HMO	Keystone Select	Yes	125% of MPFS	DRG base \$11,400 / APC 235% OPPS
ACA / Exchange	Keystone Select (narrow)	No	110% of MPFS	DRG base \$9,600 / APC 190% OPPS
Medicare Advantage	Broad	Yes	100% of MPFS	100% of IPPS / OPPS
Medicaid MCO	Keystone Select	No	— (separate agreement)	

B.1 Product Governs Rate. Where a Member's enrollment cannot be determined as of the date of service, no Product scope applies and the claim cannot be priced under this Agreement (reported as a coverage gap, not a rate conflict).

B.2 Tiering. Commercial PPO recognizes Tier 1 (Keystone-employed) and Tier 2 (affiliated) participation; Tier assignment affects Member cost-sharing, not the Allowed Amount, except where a Rate Exhibit line states otherwise.

EXHIBIT C — RATE EXHIBITS (COMMERCIAL PPO, BASE YEAR 2025)

Rates below apply to the Commercial PPO Product. Each care setting uses its own methodology. All rates are subject to the lesser-of rule (Section 4.5) and the payment policies of Exhibit E.

C.1 Professional Services (RBRVS — % of MPFS).

CPT / HCPCS	Description	Covered Entity	Basis	2025 Allowed
99213	Office visit, est., level 3	Keystone Cardiology (org)	130% MPFS	\$108.06
99213	Office visit, est., level 3	Dr. Chen (practitioner)	130% MPFS	\$108.06
99214	Office visit, est., level 4	Keystone Cardiology (org)	130% MPFS	\$152.40
93000	Electrocardiogram, complete	Keystone Cardiology (org)	130% MPFS	\$24.18
29881	Knee arthroscopy w/ meniscectomy	Keystone Cardiology (org)	130% MPFS	\$520.00

Materialization: Allowed = RVU × CMS Conversion Factor (2025 = \$32.7442) × 130%. Example 99213: 2.54 RVU × \$32.7442 × 1.30 ≈ \$108.06.

C.2 Inpatient Facility (DRG).

MS-DRG	Description	Rel. Weight	Base Rate	Case Allowed
470	Major joint replacement, lower extremity	2.00	\$12,000	\$24,000
291	Heart failure & shock w/ MCC	1.35	\$12,000	\$16,200
247	Perc. cardiovascular proc. w/ drug-eluting stent	2.10	\$12,000	\$26,400

Case Allowed = Base Rate × Relative Weight. **Outlier:** where Billed Charges exceed \$150,000 for a case, the amount above threshold pays at 65% of charges (marginal-cost factor). **Carve-outs:** solid-organ transplants and implantable devices > \$25,000 are excluded from the DRG and paid at 50% of Billed Charges. (Outlier / carve-out logic is a documented model gap.)

C.3 Outpatient Facility (APC — % of OPPS).

APC / CPT	Description	Covered Entity	Basis	2025 Allowed
5121	Level 1 diagnostic imaging	Keystone General (OP)	250% OPPS	verify
70450	CT head/brain w/o contrast	Keystone Imaging	250% OPPS	\$412.00
73030	X-ray shoulder, complete	Keystone Imaging	250% OPPS	\$96.00

Multiple-procedure reduction (MPPR) applies to imaging: subsequent procedures on the same date pay at 50% of the technical component. See Exhibit E.3.

C.4 Behavioral Health (Per-Diem).

Rev. Code	Level of Care	Covered Entity	Basis	Per-Diem
0124	Inpatient psychiatric, acute	Keystone Behavioral Health	Per day	\$850
0126	Inpatient psych, intensive	Keystone Behavioral Health	Per day	\$1,150
0912	Partial hospitalization (PHP)	Keystone Behavioral Health	Per day	\$425

C.5 Ancillary Fee Schedule (Lab / DME).

HCPCS	Description	Basis	2025 Allowed
36415	Routine venipuncture	Fee schedule	\$30.00
80053	Comprehensive metabolic panel	Fee schedule	\$58.00
J1885	Ketorolac injection, per 15mg	ASP + 6%	verify

C.6 Escalator Worked Example (Section 4.4).

CPT	2025 (base)	2026 (+3%)	2027 (+3%)
99213 @ 130% MPFS	\$108.06	\$111.30	\$114.64

Escalation compounds on the prior year's materialized rate: $\$108.06 \times 1.03 = \111.30 ; $\$111.30 \times 1.03 = \114.64 .

EXHIBIT D — VALUE-BASED & QUALITY TERMS

The fee-for-service rates in Exhibit C are subject to the following value-based adjustments. (These terms are illustrative; the current pricing engine implements fee-for-service only and treats value-based settlement as a future roadmap item.)

D.1 Quality Withhold. Plan withholds two percent (2.0%) of each Allowed Amount for professional services. The withhold is earned back based on the Health System's performance against the HEDIS and CAHPS measures in Schedule D-1, reconciled annually.

D.2 Shared Savings. For the attributed Commercial PPO population, if total cost of care is below the risk-adjusted target, the parties share savings 50/50, subject to a minimum savings rate of 2% and quality gates.

D.3 Pay-for-Performance Bonus. Up to \$250,000 annually for meeting access and patient-experience targets, paid outside the claim stream.

EXHIBIT E — PAYMENT POLICIES & CLAIM EDITS

E.1 Coding Edits (NCCI). Claims are adjudicated against CMS National Correct Coding Initiative (NCCI) procedure-to-procedure and medically-unlikely edits. Bundled and mutually-exclusive code pairs are denied or combined before pricing. (Documented model gap: NCCI is not yet implemented in the engine.)

E.2 Modifiers. Recognized modifiers adjust the Allowed Amount, including -50 (bilateral, 150%), -TC (technical component, 60%), -26 (professional component), -52 (reduced services, 50%), and -80 (assistant surgeon, 16%). Unrecognized modifiers are ignored and do not adjust the rate.

E.3 Multiple-Procedure Reduction (MPPR). For designated imaging, therapy, and surgical services, the highest-valued procedure pays at 100% and each subsequent procedure at 50%.

E.4 Timely Filing / Prompt Pay. Clean Claims must be filed within 180 days; Plan adjudicates Clean Claims within thirty (30) days. Interest accrues on late-paid Clean Claims per state law.

E.5 Coordination of Benefits. Where the Member has other coverage, this Agreement pays as primary or secondary per COB rules; the combined payment shall not exceed the Allowed Amount.

E.6 Appeals. Provider may dispute a payment determination within ninety (90) days through Plan's provider-dispute process before pursuing any external remedy.

IN WITNESS WHEREOF, the parties have executed this Agreement as of the Effective Date.

HIGHMARK HEALTH PLAN, INC.

KEYSTONE HEALTH SYSTEM, INC.

By: Patricia Nolan

Title: SVP, Provider Network

Date: _____

By: David Okafor, MD

Title: Chief Executive Officer

Date: _____

SPECIMEN / TRAINING DOCUMENT. Fictional parties and rates. Created to illustrate managed-care contract anatomy for the Matrix pricing-engine project. Not legal advice; not a real agreement.